

Manikandan Ramanathan

30 Year(s)/Male

Ref. by :

Partner : OCC HSR Layout Sec 6

Report Ref. ID : BLR5948561

Patient ID : OHP76YPK1852713

Collected : 30/12/2025 07:33 AM

Received : 30/12/2025 11:18 AM

Reported : 30/12/2025 04:39 PM



MC-6367



Test	Results	Units	Biological Reference
SEROLOGY			
Hepatitis B Surface Antigen (HbsAg) Serum,Chemiluminescent Immunoassay	0.08	S/C Units	Non-reactive: < 0.90 Reactive: ⇒ 1.00 Borderline: 0.90-1.00
Clinical Significance: The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture. A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus. Levels of HBsAg may be undetectable both in early infection and late after infection.			
VDRL Serum,Immunochromatography with Flocculation			
	Non-Reactive		Non-reactive
Clinical Significance: VDRL is a non-treponemal serological test. Biological false positive reaction can occur in: • Viral infections • Malaria • Following vaccination • Autoimmune disease such as SLE and RA • Leprosy When Biological false positives do occur, they may be of low titer (<1:8 dilution) Syphilis can be excluded in patients with biological false positive reactions by doing a treponemal(specific) serological test such as TPHA or FTA-Abs.			
Hepatitis C Virus (HCV) Antibody Serum,Chemiluminescent Immunoassay	0.01	S/C Units	Non-reactive: < 0.90 Reactive: ⇒ 1.00 Borderline: 0.90-1.00
Clinical Significance: The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture. A negative test result does not exclude the possibility of exposure to or infection with HCV. HCV antibodies may be undetectable in some stages of the infection and in some clinical conditions.			

Hepatitis B Surface Antigen (HbsAg)

Serum,Chemiluminescent Immunoassay

0.08

S/C Units

Non-reactive: < 0.90

Reactive: ⇒ 1.00

Borderline: 0.90-1.00

Clinical Significance:

The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture. A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus. Levels of HBsAg may be undetectable both in early infection and late after infection.

VDRL

Serum,Immunochromatography with Flocculation

Non-Reactive

Non-reactive

Clinical Significance:

VDRL is a non-treponemal serological test.

Biological false positive reaction can occur in:

- Viral infections
- Malaria
- Following vaccination
- Autoimmune disease such as SLE and RA
- Leprosy

When Biological false positives do occur, they may be of low titer (<1:8 dilution)

Syphilis can be excluded in patients with biological false positive reactions by doing a treponemal(specific) serological test such as TPHA or FTA-Abs.

Hepatitis C Virus (HCV) Antibody

Serum,Chemiluminescent Immunoassay

0.01

S/C Units

Non-reactive: < 0.90

Reactive: ⇒ 1.00

Borderline: 0.90-1.00

Clinical Significance:

The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture. A negative test result does not exclude the possibility of exposure to or infection with HCV. HCV antibodies may be undetectable in some stages of the infection and in some clinical conditions.

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Test	Results	Units	Biological Reference
Human Immunodeficiency Virus (HIV) 1 & 2 Serum, Chemiluminescent Immunoassay	0.22	S/C Units	Non-reactive: < 0.90 Reactive: $\Rightarrow$ 1.00

Clinical Significance:

The VITROS HIV Combo test uses 3 recombinant antigens derived from HIV-1 envelope (env 13), HIV-1 group O envelope (env 70-3) and HIV-2 envelope (env 31). These antigens detect antibodies to HIV-1 and antibodies to HIV-2 in the same test. The VITROS HIV Combo test also uses antibodies to HIV p24 antigen to enable detection of HIV p24 antigen that may be present in early seroconversion before the onset of antibody response enabling earlier diagnosis of HIV infection. The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture. Repeat testing after 3 to 6 months later is advised if clinically indicated.

Approved By

Dr. Drisya Jagadees
MBBS, MD (Pathology)
Pathologist